

Global Medical Diagnostics

Mobile Clinical Cardiopulmonary Exercise Testing (CPET)

Service area: Southern California & Arizona **Metros:** Los Angeles · Orange County · San Diego · Phoenix · Tucson

Status: Operating **Updated:** August 2026

Referrals & scheduling: (480) 806-9044 · info@globalmedicaldiagnostics.com

1. Executive Summary

Global Medical Diagnostics provides mobile, clinical cardiopulmonary exercise testing that measures how the heart, lungs, and muscles work together during exercise—bringing diagnostic-grade CPET to referring practices and patients across Southern California and Arizona.

OPERATING MODEL
Mobile clinical service at partner clinics and approved clinical sites. Clinical use only.

REPORT TURNAROUND
Structured clinical reports returned to the ordering clinician in 3 days.

PAYMENT OPTIONS
Insurance billing and \$450 cash-pay.

YEAR 1 TARGET
5,500 tests · \$1.375M revenue · ~\$522.5K net

2. The Opportunity

Unexplained shortness of breath, uncertain surgical risk, and incomplete functional workups are difficult to resolve with resting tests alone. Referring clinicians need objective cardio-pulmonary data without long hospital waits or patient travel burden.

Why current options fall short

- Resting ECG, echo, PFTs, or stress ECG without gas exchange often leave key questions unanswered
- Hospital-based CPET can mean longer waits and greater travel friction
- Fixed specialty labs have limited geographic reach across a two-state region

3. Solution

Clinical-only mobile CPET with clear referral intake, on-site supervised testing, insurance or cash-pay options, and structured reports back to the ordering clinician within 3 days.

How it works

1. Referral / order intake (indication, screening information, insurance or cash-pay path)
2. Benefits check or cash-pay confirmation (\$450), then scheduling at a mobile site
3. On-site supervised graded exercise test with metabolic gas analysis and ECG monitoring
4. Data review, interpretation, and structured clinical report
5. Results returned to the referring provider within 3 days; billing follow-through as applicable

Expected outcome: Clearer diagnosis or risk stratification, actionable functional metrics (such as VO₂ peak, ventilatory thresholds, and pattern of limitation), and a documentable next step for care.

4. Target Customers

Segment	Need	Decision trigger
Cardiologists / pulmonologists	Dyspnea differentiation; unexplained limitation workups	Need gas-exchange data after incomplete workup
Surgeons / pre-op clinics	Objective functional capacity / risk stratification	Upcoming major surgery; borderline risk
Primary care / internal medicine	Next step after normal resting tests	Persistent unexplained dyspnea
Multi-specialty groups / IDNs	Nearby diagnostic access without building a full lab	Capacity, access, or patient-leakage concerns
Cardiac / pulmonary rehab programs	Baseline and progress metrics	Outcome tracking or program expansion

Positioning is clinical-only. Sports performance / athlete consumer testing is not part of the current offer.

5. Services and Pricing

Service	Description	Price model
Clinical CPET (core)	Diagnostic CPET with ECG, gas exchange, and clinical report	Insurance or \$450 cash-pay
Pre-op / risk-stratification CPET	Protocol aligned to surgical risk questions	Insurance or cash-pay
Rehab baseline / follow-up CPET	Pre/post functional testing for clinical programs	Insurance, cash-pay, or program agreement
Mobile on-site clinic day	Testing block at a partner practice or approved site	Per-test billing and/or site scheduling agreement

- **Cash-pay:** \$450 per clinical CPET

- **Insurance:** Eligibility/authorization as required; patient responsibility per plan
- **Planning revenue assumption:** \$250 blended collected revenue per test for multi-year projections

6. Market and Differentiation

SERVICE FOOTPRINT

Southern California and Arizona, with priority metros in Los Angeles, Orange County, San Diego, Phoenix, and Tucson.

COMPETITIVE SET

Hospital cardiopulmonary labs, health-system outpatient diagnostics, and fixed independent labs.

Differentiation

- Mobile clinical delivery across five priority metros
- Clinical-only focus as a trusted diagnostic partner
- Insurance billing plus published \$450 cash-pay option
- 3-day report turnaround
- Referring-office convenience—testing brought closer to practice and patient

7. Go-to-Market

- Provider and patient website (indications, how to refer, prep, metros served)
- Direct outreach to cardiology, pulmonology, pre-op, and multi-specialty groups
- Partner clinic “mobile day” scheduling
- Phone and email intake: (480) 806-9044 · info@globalmedicaldiagnostics.com
- Sample reports, indication one-pagers, and educational outreach

Messaging pillars

1. Clinical CPET where your patients are—mobile in LA, OC, San Diego, Phoenix, and Tucson
2. Reports back in 3 days
3. Insurance billing or \$450 cash-pay

8. Operations

Workflow: Order/referral → clinical screening → insurance or cash-pay clearance → schedule mobile site → on-site test → quality review → interpretation → report to ordering clinician within 3 days → billing follow-through

- **Availability:** By scheduled mobile clinic days in priority metros
- **Core systems:** Mobile metabolic cart, ergometer and/or treadmill, ECG, SpO₂, scheduling/referral intake, secure report delivery, site emergency response plan
- **Quality & safety:** Contraindication screening, supervised testing, calibration logs, HIPAA-compliant records, site suitability criteria

- **Support:** Referring-office coordination by phone/email; patient prep guidance; escalation path for urgent findings

9. Financial Plan

Unit economics (per test)

Line item	Amount
Blended revenue	\$250
Technician payroll	\$30
Medical supplies	\$15
Interpretation professional fees	\$50
Transportation	\$10
Reports	\$10
Insurance	\$5
Total variable cost	\$120
Contribution margin	\$130
Fixed overhead (allocated)	\$35
Fully loaded cost	\$155
Net margin per test	\$95 (38%)

Five-year projection

Year	Tests	Revenue	Variable	Fixed OH	Net
Year 1	5,500	\$1,375,000	\$660,000	\$192,500	\$522,500
Year 2	8,500	\$2,125,000	\$1,020,000	\$297,500	\$807,500
Year 3	12,000	\$3,000,000	\$1,440,000	\$420,000	\$1,140,000
Year 4	16,000	\$4,000,000	\$1,920,000	\$560,000	\$1,520,000
Year 5	20,000	\$5,000,000	\$2,400,000	\$700,000	\$1,900,000
5-Year Total	62,000	\$15,500,000	\$7,440,000	\$2,170,000	\$5,895,000

Year 1 monthly run rate

Item	Annual	Monthly
Tests	5,500	~458
Revenue	\$1,375,000	~\$114,583
Total variable costs	\$660,000	\$55,000
Contribution	\$715,000	~\$59,583
Fixed overhead	\$192,500	~\$16,042
Net	\$522,500	~\$43,542

\$250 is a blended planning/collections assumption per test and is not the same as the \$450 cash-pay list price. Fixed overhead is modeled at \$35 per test; if true fixed costs do not scale linearly with volume, later-year net results may be higher.

10. Milestones

Timeframe	Milestone	Success metric
30 days	Advance website and referral materials	Contact, metros, 3-day turnaround, and \$450 cash-pay clearly published
90 days	Deepen referring base in five metros	Improved route density; progress toward ~458 tests/month
12 months	Achieve Year 1 commercial plan	5,500 tests; \$1.375M revenue; ~\$522.5K net; 3-day report SLA held

11. Risks and Mitigations

Risk	Likelihood	Impact	Mitigation
Thin routes / high travel time	High	High	Cluster clinic days by metro; minimum-volume site rules
Collections below \$250 blended plan	High	High	Authorization discipline, denial reduction, cash-pay mix management
Capacity lag vs volume ramp	Medium	High	Hire/equipment plan tied to quarterly test counts
Partner site not test-ready	Medium	High	Written site checklist before booking
Equipment failure on the road	Medium	High	Service plan, spare critical parts, backup reschedule protocol
Report turnaround beyond 3 days	Medium	Medium	Queue tracking; interpreter backup coverage
Clinical incident off main campus	Low	Very high	Screening, trained team, site emergency plan, insurance

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